

Derbyshire Shared Care Pathology Guidelines

PSA in general practice

This guideline is aligned with the NICE CKS "How should I assess a person with suspected prostate cancer?"

<https://cks.nice.org.uk/topics/prostate-cancer/diagnosis/assessment/#information-advice-on-psa-testing>

- Use the 'Information and advice on PSA testing' in the link above to provide appropriate information to enable the person to make an informed choice about testing
- Men with African heritage tend to have more aggressive prostate cancer and should be actively encouraged to be tested

Before having a PSA test, men should not have:

- An active urinary infection (PSA may remain raised for many months)
- Ejaculated in the previous 48 hours
- Exercised vigorously in the previous 48 hours
- Had a prostate biopsy in the previous 6 weeks

**Urinary Tract Infection can elevate PSA; wait for 6 weeks if patient has UTI.
Refer any malignant feeling prostate regardless of the PSA result.**

PSA Levels

PSA is not tumour specific in the prostate. PSA levels can increase due to a number of other factors:

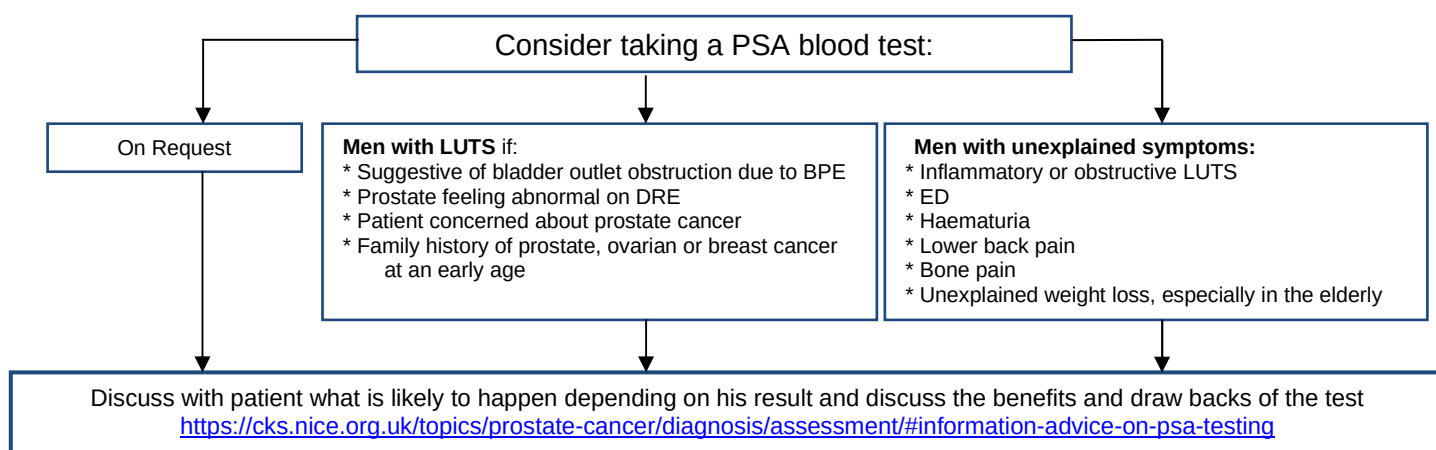
- BPE
- Older age – PSA levels normally increase with age
- Prostatitis – infection or inflammation of the prostate gland
- Ejaculation – may increase PSA levels for a short time

**For patients on 5-alpha-reductase inhibitors (Finasteride, Dutasteride) a fall in PSA would be expected (median 50% reduction after 6 months).
A rise of PSA of 2µg/L or more from the nadir when on Finasteride or Dutasteride should be considered significant.**

Lower PSA levels can also be seen in:

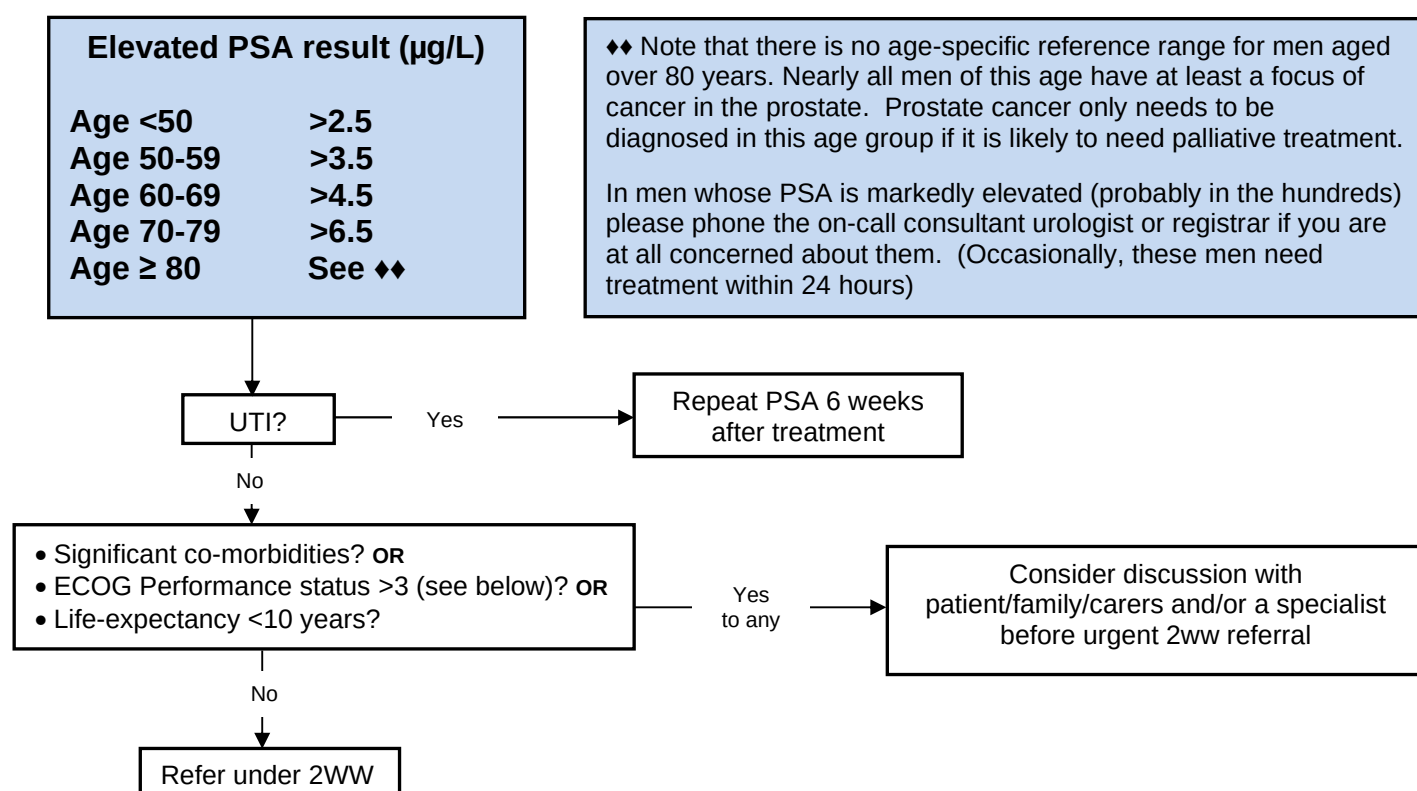
- Patients on aspirin, statin, thiazide diuretic
- Obese men

Diagnostic evaluation should consider the PSA level together with the man's history of comorbidities, ethnicity, family history and abnormal DRE findings prior to biopsy.



Special considerations

- Defer PSA by 6 weeks if signs or symptoms of infection or inflammation
- **Repeat PSA if UTI is proven, wait for 6 weeks**
- NOTE: DRE does not affect the PSA
- **Refer any malignant feeling prostate regardless of the PSA result**



ECOG Performance Status Assessment

0 – Asymptomatic (Fully active, able to carry on all pre-disease activities without restriction)

1 – Symptomatic but completely ambulatory (Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature. For example, light housework, office work)

2 – Symptomatic, <50% in bed during the day (Ambulatory and capable of all self-care but unable to carry out any work activities. Up and about more than 50% of waking hours)

3 – Symptomatic, >50% in bed, but not bedbound (Capable of only limited self-care, confined to bed or chair 50% or more of waking hours)

4 – Bedbound (Completely disabled. Cannot carry on any self-care. Totally confined to bed or chair)

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	V3 Jul 2023	Miss Kate Linton, Mr S Williams, Dr P Blackwell, Mrs H Seddon	Routine Review
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Approval	Date	Name(s)	Designation
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